ADR IDENTIFICATION BASICS

Task Report: Identifying a Possible Adverse Drug Reaction (ADR)

1. Patient Scenario

Patient: Female, 45 years.
Reported symptoms: Rash, itching, nausea, and dizziness.
Medicine mentioned: Amoxicillin, an antibiotic.
Information to confirm: When amoxicillin was started, when symptoms began, dose and last dose, other medicines, allergy history, and whether symptoms changed after each dose.

2. Assessment

Suspected medicine: Amoxicillin. Rash and itching can be signs of a medicine-related allergic reaction. Nausea may also occur as an adverse effect, while dizziness is nonspecific and needs assessment. Symptoms alone do not prove that amoxicillin caused the reaction; timing and other possible causes must be checked.

3. Timing and Causality

Establish whether symptoms began after amoxicillin was started and how soon after a dose they appeared. Review previous exposure, known penicillin allergy, concurrent medicines, the underlying infection, and other possible explanations. A qualified healthcare professional should complete a formal causality assessment using all available evidence.

4. Recommended Response and Safety

Because rash and itching occurred alongside dizziness, prompt medical assessment is appropriate. If there is difficulty breathing, wheezing, swelling of the lips, tongue or throat, fainting, collapse, or rapidly worsening symptoms, seek emergency care immediately. Do not take another dose until a prescriber or qualified clinician has advised what to do. Document the suspected medicine, dose, dates and times, symptoms, treatment, outcome, and relevant allergy history. Report the suspected ADR through the appropriate local pharmacovigilance system according to workplace policy.

5. Conclusion

Amoxicillin is the suspected drug in this scenario, but causality cannot be confirmed from the available screenshot details alone. The symptom timeline and full medication history are essential. Patient safety comes first: assess for signs of a serious allergic reaction, document the event, and ensure appropriate clinical follow-up.

Educational task report only. This is not a definitive diagnosis or a substitute for assessment by a healthcare professional.